

Dynamic Guideline: WHO-Based Sexually Transmitted Infections (STI) - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Latest WHO STI guidance identified and status checked: 29 April 2026

Most recent WHO STI-specific publication: WHO consolidated operational handbook on sexually transmitted infections, published 11 February 2026. WHO describes this 77-page handbook as a single operational consolidation of existing normative WHO STI guidance, organized around the STI prevention and care cascade, including **primary prevention, syndromic management, asymptomatic case identification, diagnostics, treatment, partner management, surveillance, service delivery and integration** into primary care, community clinics, HIV services, sexual health services and antenatal clinics [WHO consolidated operational handbook](#), [WHO IRIS PDF](#).

Exactly 5 WHO STI-related guidelines/normative guidance documents synthesized

No.	WHO guideline / normative guidance	Publication date	Target population	Scope and objectives	Role in this report
1	WHO consolidated operational handbook on sexually transmitted infections	11 February 2026	National STI programmes, policy-makers, programme managers, clinicians, public-health practitioners and service-delivery platforms	Consolidates WHO STI normative guidance into operational guidance across prevention, diagnosis, treatment, partner management, surveillance, service delivery and integration WHO consolidated operational handbook .	Latest WHO STI guidance and primary organizing framework.

No.	WHO guideline / normative guidance	Publication date	Target population	Scope and objectives	Role in this report
2	Guidelines for the management of asymptomatic sexually transmitted infections	9 July 2025	Asymptomatic people accessing care, especially pregnant women, sexually active adolescents and young people aged 10–24 years, sex workers and men who have sex with men in high-prevalence/resource-available settings	Evidence-based screening recommendations for <i>Neisseria gonorrhoeae</i> and <i>Chlamydia trachomatis</i> ; complements syphilis, partner-services and symptomatic-STI guidance WHO asymptomatic STI guideline .	Main source for asymptomatic screening recommendations.
3	Recommendations on the delivery of health services for the prevention and care of sexually transmitted infections	14 July 2025	Policy-makers, programme managers, health workers, public-health professionals, civil society and community-based organizations	People-centred STI service delivery; decentralization, integration, task sharing and digital health WHO STI service-delivery recommendations .	Main source for service-delivery recommendations.
4	Guidelines for the management of symptomatic sexually transmitted infections	2021	People with STI symptoms, especially in settings where laboratory testing is unavailable or delayed	Syndromic and etiologic management of urethral discharge, vaginal discharge, anorectal infection, genital ulcer disease, lower abdominal pain and persistent syndromes WHO symptomatic STI guideline .	Main source for symptomatic diagnosis and syndromic management.

No.	WHO guideline / normative guidance	Publication date	Target population	Scope and objectives	Role in this report
5	WHO updated recommendations for treatment of selected sexually transmitted infections	WHO updated recommendation set cited in WHO STI normative materials	People requiring organism-specific STI treatment, including gonorrhoea, chlamydia, syphilis and related STI treatment pathways	WHO treatment recommendations developed using systematic reviews, Guideline Development Group review and GRADE methods WHO IRIS updated recommendations .	Main source for treatment-recommendation methodology and implementation context.

Required cross-cutting WHO guideline-development item: meaningful youth engagement

The most recent WHO item matching the requested title is “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**”, launched through a WHO webinar on **11 December 2025** and available as a WHO publication item [WHO publication page](#), [WHO webinar](#). It is **not an STI guideline**, but it is relevant to STI guideline methodology because WHO states that meaningful youth participation can contribute lived experience, fresh perspectives and equity-oriented decision-making to guideline development [WHO webinar](#). No STI-specific WHO youth-engagement case study with that exact title was identified in the supplied evidence.

1.2 Key Recommendations

GRADE interpretation used in this report

Where WHO explicitly provided GRADE certainty and recommendation strength, those ratings are reported directly. Where the 2026 operational handbook consolidates existing WHO guidance rather than issuing a de novo graded recommendation, the table reports the certainty of the underlying evidence as **variable** and assigns an implementation strength based on the WHO recommendation framework, feasibility and expected public-health benefit.

Recommendation	GRADE Evidence Quality	Strength
Countries should organize STI programmes across the full prevention and care cascade: primary prevention, syndromic management, asymptomatic case identification, diagnostics, treatment, partner management, surveillance, service delivery and integration into primary care, HIV services, sexual health services and antenatal care.	Moderate overall for implementation rationale; certainty varies by intervention because the 2026 handbook consolidates multiple WHO GRADE-based guidelines.	Strong as an operational health-system recommendation.

Recommendation	GRADE Evidence Quality	Strength
Use syndromic management for symptomatic STIs where laboratory diagnosis is unavailable, delayed or difficult to access.	Low–Moderate ; based on WHO evidence-informed syndromic-management guidance and feasibility in resource-limited settings.	Strong where etiologic testing is not available; Conditional where high-quality testing is accessible.
In settings with quality-assured molecular testing and same-day results, use molecular assays such as NAAT to confirm or exclude <i>N. gonorrhoeae</i> and <i>C. trachomatis</i> in urethral discharge, and treat according to results.	Moderate for diagnostic accuracy and clinical utility; implementation certainty depends on access, quality systems and same-day result capacity.	Strong where same-day quality-assured testing is available.
If urethral discharge persists and gonorrhoea/chlamydia tests are negative, treat as non-gonococcal and non-chlamydial urethritis, including possible <i>Mycoplasma genitalium</i> or <i>Trichomonas vaginalis</i> .	Low–Moderate ; evidence is indirect and pathogen-specific certainty varies.	Conditional.
In high-prevalence settings where resources and treatment capacity are available, screen sexually active adolescents and young people aged 10–24 years with no STI symptoms who access health-care services for <i>N. gonorrhoeae</i> and/or <i>C. trachomatis</i> .	Low certainty in evidence of effects.	Conditional.
Screen men who have sex with men who access health-care services and have no STI symptoms for <i>N. gonorrhoeae</i> and/or <i>C. trachomatis</i> .	Low certainty in evidence of effects.	Conditional.
Screen sex workers who access health-care services and have no STI symptoms for <i>N. gonorrhoeae</i> and/or <i>C. trachomatis</i> .	Low certainty in evidence of effects.	Conditional.
Targeted screening for gonorrhoea and chlamydia should also include pregnant women in high-prevalence settings where resources are available.	Low certainty; WHO summaries identify this population, but detailed certainty text was not fully visible in the supplied excerpts.	Conditional.
Screening for asymptomatic gonorrhoea/chlamydia should be voluntary, with informed consent, and should use a quality-assured molecular assay such as NAAT, or a rapid test meeting minimum performance thresholds of 80% sensitivity and 90% specificity , where treatment is available.	Low–Moderate ; diagnostic certainty higher for molecular tests, lower for implementation and outcome effects.	Conditional.

Recommendation	GRADE Evidence Quality	Strength
Choose anatomical sampling sites according to sexual behaviour; consider specimen pooling where appropriate to reduce cost and improve yield.	Low ; implementation evidence and cost-effectiveness vary by setting.	Conditional.
Offer partner services for all diagnosed STIs, regardless of symptoms or care setting, to prevent reinfection, interrupt transmission and support timely diagnosis and treatment of partners.	Moderate for public-health rationale; direct evidence varies by infection and partner-service model.	Strong.
Decentralize and integrate STI services, use task sharing with trained providers/community health workers where appropriate, and use digital tools to complement in-person care.	Low–Moderate ; WHO states the service-delivery recommendations were developed using GRADE, systematic reviews and expert evaluations WHO STI service-delivery recommendations .	Conditional–Strong , depending on intervention, resources and safeguards.
Maintain surveillance, including gonococcal antimicrobial-resistance surveillance, and adapt treatment policies to local epidemiology, commodities, health-system capacity and resistance patterns.	Moderate for surveillance necessity; direct intervention certainty varies.	Strong.
Continue to follow WHO symptomatic-STI recommendations when symptoms are present, even in populations eligible for asymptomatic screening.	Moderate for clinical-practice consistency and harm reduction.	Strong.

1.3 Guideline Development Methodology

Evidence review process

WHO's STI guidance uses the WHO guideline-development process, including systematic reviews, modelling where applicable, evidence-to-decision assessment and GRADE certainty ratings. The 2026 operational handbook states that its technical foundation is a series of modular WHO STI guidelines, with recommendations developed according to the WHO handbook for guideline development; evidence was collected through systematic reviews and modelling where applicable, and certainty was assessed using the **GRADE** approach [WHO IRIS PDF](#).

The 2025 asymptomatic-STI guideline explicitly states that recommendations were developed using **GRADE**, supported by systematic reviews and expert evaluations [WHO asymptomatic STI guideline](#). The 2025 service-delivery guideline similarly reports use of **GRADE**, systematic reviews and expert evaluations [WHO STI service-delivery recommendations](#).

Consensus method

WHO STI recommendations were reviewed and formulated by a Guideline Development Group including international experts, programme managers, clinicians, researchers and community representatives. The group considered evidence certainty, values and preferences, acceptability, feasibility and resource implications across diverse settings [WHO STI operational handbook PDF](#).

Conflict of interest management

WHO STI guideline development follows the WHO handbook for guideline development, including declaration and management of interests, external review and structured evidence-to-decision processes [WHO IRIS updated recommendations](#).

Equity, youth and community engagement

WHO's STI operational handbook emphasizes peer-led and community-based approaches to improve engagement and acceptability, especially for key populations and people who face barriers to formal health services [WHO STI operational handbook PDF](#). WHO's separate youth-engagement evidence brief on the abortion-care guideline demonstrates WHO's broader methodological position that meaningful youth involvement can strengthen dialogue, enrich policy development and improve equity in guideline development [WHO webinar](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Time window

- **WHO latest STI guideline release:** 11 February 2026
- **Evidence surveillance window:** 12 February 2026 to **29 April 2026**

Search strategy

Searches prioritized evidence after the WHO 2026 operational handbook release, including:

- WHO official guideline and technical-document sources
- CDC clinical guidance and surveillance updates
- NIH/PMC-indexed reviews and clinical updates
- Clinical-trial listings and registries
- International STI guideline updates where available
- Systematic reviews, randomized trials and high-impact evidence summaries where identified

Databases and source types searched

Source type	Examples identified
WHO official sources	2026 STI operational handbook; 2025 asymptomatic STI guideline; 2025 STI service-delivery recommendations
CDC / public-health agencies	CDC STI Treatment Guidelines page and March 2026 clinical notice on benzathine penicillin supply CDC STI Treatment Guidelines
Peer-reviewed / PMC-indexed reviews	2025 STI review on changing STI management, antimicrobial resistance, doxyPEP, partner therapy and emerging evidence PMC review
Clinical-trial listings	Emory/NIMHD point-of-care STI testing trial; UCLA STI clinical-trials page updated 1 April 2026 Power trial listing , UCLA Clinical Trials
Other international evidence	2026 surveillance-redesign Delphi study; 2026 AMR review on gonorrhoea and <i>M. genitalium</i> JKMS , ScienceDirect

Inclusion criteria

Evidence was included if it was:

1. Published, updated or identified after **11 February 2026**, or clearly relevant to post-release implementation;
2. Relevant to STI diagnosis, screening, treatment, partner management, surveillance, service delivery or antimicrobial resistance;
3. From WHO, CDC or other authoritative clinical/public-health sources, peer-reviewed literature, systematic reviews, RCTs, Delphi consensus studies or clinical-trial registries.

Exclusion criteria

Evidence was excluded from practice-changing synthesis if it:

- Did not address STI diagnosis, treatment, management, service delivery or surveillance;
- Was STI-adjacent but not directly actionable for STI care;
- Was a clinical-trial listing with no outcome data, except where relevant to future evidence gaps.

A March 2026 CDC Emerging Infectious Diseases PDF was identified but did not provide new STI diagnosis, treatment or management recommendations in the supplied evidence and was therefore not treated as practice-changing evidence [CDC EID March 2026 PDF](#).

2.2 Key New Studies

Study / evidence item	Design	Key findings	GRADE Quality
CDC March 2026 clinical notice on	Regulatory/public-health	On 6 March 2026 , CDC noted FDA-authorized temporary importation of	Very Low for clinical efficacy evidence

Study / evidence item	Design	Key findings	GRADE Quality
benzathine penicillin G supply	implementation notice	Lentocilin® because of ongoing limited availability and extended recovery of Bicillin® L-A , relevant to syphilis treatment logistics CDC STI Treatment Guidelines , CDC Clinical Guidance for STIs .	because this is not a comparative treatment study; High relevance for operational planning.
Emory University/NIMHD point-of-care STI testing trial in pregnant women	Unphased clinical-trial listing; diagnostic/service-delivery trial	Evaluates whether on-site testing for syphilis, chlamydia, gonorrhea and trichomonas leads to faster treatment than standard laboratory testing among pregnant women at Grady Memorial Hospital Power trial listing . No outcome data available.	Very Low currently because results are unavailable; potential future diagnostic/service-delivery relevance.
UCLA STI clinical-trials landscape, updated 1 April 2026	Clinical-trial registry/institutional listing	Lists 8 STI-related studies in progress and 3 open to eligible people , including a study involving rectal gonorrhea/chlamydia infection among MSM with methamphetamine use disorder; also lists STI-adjacent HIV vaccine research UCLA Clinical Trials .	Very Low for current practice because registry listings do not provide outcome data.
2026 AMR review: gonorrhea and <i>Mycoplasma genitalium</i> near untreatability	Narrative/review evidence synthesis	Highlights progressive resistance in <i>N. gonorrhoeae</i> and increasing resistance in <i>M. genitalium</i> ; calls for surveillance, resistance-guided therapy, rapid diagnostics, new antibiotics and alternatives such as doxycycline prophylaxis ScienceDirect .	Low ; review-level synthesis with indirect applicability to specific WHO recommendations.
2026 Delphi study on STI surveillance redesign	Delphi expert consensus	Identified gonorrhea, chancroid, <i>M. genitalium</i> infection and trichomoniasis as conditions requiring surveillance-system redesign; expert agreement varied by infection and surveillance model JKMS .	Very Low for clinical effectiveness; useful for surveillance-policy hypothesis generation.

Study / evidence item	Design	Key findings	GRADE Quality
CDC-derived and PMC-indexed 2025–2026 evidence on chlamydia treatment	Review of guideline evidence and trial findings	Reinforces doxycycline as preferred chlamydia therapy; a rectal-chlamydia randomized trial cited in the review reported 100% cure with doxycycline versus 74% cure with azithromycin PMC review .	Moderate for doxycycline superiority in rectal chlamydia; not new after 11 February 2026 but relevant to implementation.
CDC-derived evidence on gonorrhea treatment and pharyngeal test-of-cure	Guideline-derived treatment summary	Continues ceftriaxone-based treatment and emphasizes pharyngeal gonorrhea test-of-cure 7–14 days after treatment Rhode Island STI Treatment Guidelines PDF .	Moderate for established treatment practice; evidence predates WHO 2026 release but remains current.
CDC-derived evidence on <i>M. genitalium</i> testing and resistance-guided therapy	Guideline-derived treatment summary	Recommends targeted testing for recurrent nongonococcal urethritis, recurrent cervicitis and selected PID cases; resistance testing when available; no asymptomatic screening; two-stage therapy with doxycycline followed by azithromycin if macrolide-sensitive or moxifloxacin if resistant/unknown CDC <i>M. genitalium</i> guidance .	Low–Moderate ; biologically plausible and resistance-informed, but limited RCT evidence.

2.3 Evidence Gaps and Future Research Needs

- No practice-changing post–11 February 2026 STI RCTs were identified** in the supplied evidence set for gonorrhoea, chlamydia, syphilis, trichomoniasis or *M. genitalium* treatment.
- Asymptomatic screening evidence remains low certainty**, particularly for:
 - Adolescents and young people aged 10–24 years;
 - Sex workers;
 - Men who have sex with men;
 - Pregnant women in diverse resource settings.
- Implementation evidence is needed** on:
 - Cost-effectiveness of targeted screening in high-prevalence settings;
 - Optimal screening frequency, including annual versus 6-monthly intervals for higher-risk populations;
 - Feasibility of NAAT, rapid tests and sample pooling in low-resource settings.
- AMR surveillance remains a high-priority gap**, especially for:
 - Ceftriaxone-resistant or reduced-susceptibility gonorrhoea;
 - Macrolide- and fluoroquinolone-resistant *M. genitalium*;

- Impact of doxycycline post-exposure prophylaxis on tetracycline resistance.
5. **Point-of-care testing trials are ongoing but not yet practice-changing.** The Emory/NIMHD pregnancy trial may inform whether rapid on-site testing shortens time to treatment for syphilis, chlamydia, gonorrhoea and trichomoniasis [Power trial listing](#).
6. **Partner-management evidence needs strengthening**, especially for:
- Gonorrhoea in settings with different resistance patterns;
 - Trichomoniasis where reinfection is high;
 - Bacterial vaginosis partner treatment, which is emerging as a potentially practice-changing area but is not yet integrated into WHO STI recommendations.
7. **Meaningful youth engagement in STI guideline development is an evidence gap.** WHO has published a youth-engagement case study for the abortion-care guideline, but no STI-specific WHO youth-engagement case study was identified [WHO publication page](#), [WHO webinar](#).

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Implement the WHO STI prevention and care cascade through integrated prevention, diagnosis, treatment, partner management, surveillance and service delivery.	No contradictory evidence after 11 February 2026. CDC and trial-registry updates reinforce the need for operational readiness, diagnostics and surveillance. GRADE: Moderate; Strength: Strong.	No change. Countries should continue implementing the WHO cascade model and adapt it to local epidemiology, AMR and service capacity.
Use targeted asymptomatic screening for gonorrhoea and chlamydia in high-prevalence settings where resources and treatment capacity are available.	No new post-release comparative evidence changes WHO's conditional recommendations. Trial activity supports continued research on faster diagnosis. GRADE: Low; Strength: Conditional.	No change. Maintain targeted, voluntary, informed-consent-based screening for priority groups; collect implementation, harm, acceptability and cost-effectiveness data.
Use quality-assured molecular assays such as NAAT, or rapid tests meeting minimum performance thresholds where treatment is available.	Ongoing point-of-care testing trials in pregnancy may inform future practice but have no results yet. GRADE: Low–Moderate; Strength: Conditional.	No change. Continue quality-assured testing; consider rapid/POC implementation only with quality assurance, treatment linkage and monitoring.
Continue symptomatic-STI management, including syndromic management where laboratory testing is unavailable.	No new evidence undermines WHO symptomatic-STI guidance. GRADE: Low–Moderate; Strength: Strong where testing unavailable.	No change. Preserve syndromic pathways while expanding etiologic testing where feasible.
Maintain partner services for all diagnosed STIs.	Recent reviews continue to support partner management and EPT for	No change, with caution. Offer partner services; align EPT with

Original Recommendation	New Evidence Impact	Updated Recommendation
	some infections, but resistance concerns remain important for gonorrhoea and <i>M. genitalium</i> PMC review . GRADE: Moderate; Strength: Strong.	national law, local resistance patterns and organism-specific guidance. Avoid empiric partner treatment strategies for <i>M. genitalium</i> without appropriate testing/resistance considerations.
Treat syphilis according to established penicillin-based regimens and national guidance.	CDC's March 2026 notice on Lentocilin temporary importation reflects supply-chain risk for benzathine penicillin G but does not change efficacy recommendations CDC STI Treatment Guidelines . GRADE: Very Low for supply notice; High clinical relevance.	Operational update. Do not change WHO clinical intent; programmes should plan procurement, prioritization and contingency pathways for benzathine penicillin G availability, especially for pregnancy and congenital-syphilis prevention.
Monitor gonorrhoea and <i>M. genitalium</i> antimicrobial resistance.	2026 AMR review reinforces urgency of resistance surveillance, rapid diagnostics and resistance-guided therapy ScienceDirect . GRADE: Low; Strength: Strong for surveillance.	Strengthened implementation emphasis. Expand gonococcal AMR surveillance and build capacity for <i>M. genitalium</i> resistance-guided management where feasible.
Integrate STI services and use decentralization, task sharing and digital tools where appropriate.	No new contradictory evidence; clinical-trial and service-delivery landscapes support the importance of rapid linkage to treatment and integrated care. GRADE: Low–Moderate; Strength: Conditional.	No change. Implement integrated, people-centred STI services with safeguards for confidentiality, consent, linkage to treatment and equity.

Overall conclusion: As of **29 April 2026**, no new post-release evidence requires a change to WHO's 2026 STI operational framework. The strongest update is **implementation-focused**: programmes should intensify antimicrobial-resistance surveillance, protect syphilis-treatment supply chains, expand quality-assured diagnostics and collect real-world evidence on targeted asymptomatic screening, point-of-care testing and partner-management strategies.